

For the use of a Registered Medical Practitioner only

PRESCRIBING INFORMATION

ETROBAX

(Etoricoxib Tablets 60/90/120 mg)

ETROBAX 60 mg:

Each film coated tablet contains

Etoricoxib.....60 mg

Active Ingredient: Etoricoxib

Inactive Ingredients: Cellulose microcrystalline, calcium hydrogen phosphate anhydrous, croscarmellose sodium, magnesium stearate, opadry II green 35K510000

Opadry composition: Lactose monohydrate, HPMC 2910/hypromellose, titanium dioxide, triacetin, FD&C Blue #2/indigo carmine aluminum lake, iron oxide yellow

ETROBAX 90 mg:

Each film coated tablet contains

Etoricoxib.....90 mg

Active Ingredient: Etoricoxib

Inactive Ingredients: Cellulose microcrystalline, calcium hydrogen phosphate anhydrous, croscarmellose sodium, magnesium stearate, opadry II white 35K580003

Opadry composition: Lactose monohydrate, HPMC 2910/hypromellose, titanium dioxide, triacetin,

ETROBAX 120 mg:

Each film coated tablet contains

Etoricoxib.....120 mg

Active Ingredient: Etoricoxib

Inactive Ingredients: Cellulose microcrystalline, calcium hydrogen phosphate anhydrous, croscarmellose sodium, magnesium stearate, opadry II green 32K510018

Opadry composition: Lactose monohydrate, HPMC 2910/hypromellose, titanium dioxide, triacetin, FD&C Blue #2/indigo carmine aluminum lake, iron oxide yellow

PRODUCT DESCRIPTION

ETROBAX 60 mg:

Green to dark green, film coated, round biconvex tablets, E60 debossed on one side and plain on other side

ETROBAX 90 mg:

White to off white, film coated, round biconvex tablets, E90 debossed on one side and plain on other side

ETROBAX 120 mg:

Light green to pale green, film coated round biconvex tablets, E120 debossed on one side and plain on other side

PHARMACODYNAMIC AND PHARMACOKINETIC PROPERTIES

- Pharmacodynamics

Mechanism of Action

Etoricoxib is a non-steroidal anti-inflammatory drug (NSAID) that exhibits anti-inflammatory, analgesic and antipyretic activities. Etoricoxib is a potent, orally active, highly selective cyclooxygenase-2 (COX-2) inhibitor within and above the clinical dose range. Two isoforms of cyclooxygenase have been reported: cyclooxygenase-1 (COX-1) and cyclooxygenase-2 (COX-2). COX-1 is responsible for prostaglandin-mediated normal physiologic functions such as gastric cycloprotection and platelet aggregation. Inhibition of COX-1 by nonselective NSAIDs has been reported to be associated with gastric damage and platelet inhibition. COX-2 has been reported to be primarily responsible for the synthesis of prostanoid mediators of pain, inflammation, and fever. Selective inhibition of COX-2 by etoricoxib decreases these clinical signs and symptoms with decreased GI toxicity and without effects on platelet function.

Etoricoxib has been reported to produce dose-dependent inhibition of COX-2 without inhibition of COX-1 at doses up to 150 mg daily.

Platelet Function

It has been reported that multiple doses of etoricoxib up to 150 mg administered daily up to nine days had no effect on bleeding time. Similarly, bleeding time was not reported to be altered with etoricoxib 250 or 500 mg. There were no reports of inhibition of *ex vivo* arachidonic acid or collagen-induced platelet aggregation at steady state with doses of etoricoxib up to 150 mg. This information is consistent with the COX-2 selectivity of etoricoxib.

- Pharmacokinetics

Orally administered etoricoxib is well absorbed. The pharmacokinetics of etoricoxib is linear across the clinical dose range. The pharmacokinetic parameters of etoricoxib are presented below.

Peak plasma concentration (C_{max})	3.6 mcg/mL
Time to reach peak plasma concentration (T_{max})	1 hour
Area under the plasma concentration time curve	37.8 mcg•hr/mL

from 0 to 24h (AUC _{0-24h})	
Oral bioavailability	100 %

A standard meal had no clinically meaningful effect on the extent or rate of absorption of a dose of etoricoxib 120 mg.

The pharmacokinetics of etoricoxib were reported to be similar (comparable AUC, C_{max} within approximately 20 %) when administered alone, with a magnesium/ aluminium hydroxide antacid, or a calcium carbonate antacid.

Etoricoxib is approximately 92 % bound to human plasma protein over the range of concentrations of 0.05 to 5 mcg/mL. The volume of distribution at steady state (V_{dss}) is approximately 120 L in humans. It is reported that etoricoxib crosses the placenta in rats and rabbits, and the blood-brain barrier in rats.

Etoricoxib is extensively metabolized with <1 % of a dose recovered in urine as the parent drug. The major route of metabolism to form the 6'-hydroxymethyl derivative is catalyzed by cytochrome P450 (CYP) enzymes. Five metabolites have been reported in man. The principal metabolite is the 6'-carboxylic acid derivative of etoricoxib formed by further oxidation of the 6'-hydroxymethyl derivative. These principal metabolites either demonstrate no measurable activity or are only weakly active as COX-2 inhibitors. None of these metabolites inhibit COX-1.

Elimination of etoricoxib occurs almost exclusively through metabolism followed by renal excretion. Approximately 70% of the administered dose is reported to get excreted in urine and 20 % in faeces, mostly as metabolites. Less than 2 % was reported to be recovered as unchanged drug.

INDICATIONS

Etoricoxib is indicated for:

- Acute and chronic treatment of the signs and symptoms of osteoarthritis (OA) and rheumatoid arthritis (RA)
- Treatment of ankylosing spondylitis (AS)
- Treatment of acute gouty arthritis
- Treatment of acute pain, including that related to primary dysmenorrhea and minor dental procedures

The decision to prescribe a selective COX-2 inhibitor should be based on an assessment of the individual patient's overall risks.

DOSE AND METHOD OF ADMINISTRATION

ETROBAX is administered orally. ETROBAX may be taken with or without food. ETROBAX should be administered for the shortest duration possible and the lowest effective daily dose should be used.

Osteoarthritis

The recommended dose is 30 mg or 60 mg once daily.

Rheumatoid arthritis

The recommended dose is 60 mg once daily. In some patients with insufficient relief from symptoms, an increased dose of 90 mg once daily may increase efficacy. Once the patient is clinically stabilised, down-titration to a 60 mg once daily dose may be appropriate. In the absence of an increase in therapeutic benefit, other therapeutic options should be considered.

Ankylosing spondylitis

The recommended dose is 60 mg once daily. In some patients with insufficient relief from symptoms, an increased dose of 90 mg once daily may increase efficacy. Once the patient is clinically stabilised, down-titration to a 60 mg once daily dose may be appropriate. In the absence of an increase in therapeutic benefit, other therapeutic options should be considered.

Acute Pain

In the following acute painful conditions, ETROBAX should be used only for the acute symptomatic period, limited to a maximum of 8 days treatment:

- Acute Gouty Arthritis

The recommended dose is 120 mg once daily.

- Primary Dysmenorrhea

The recommended dose is 120 mg once daily.

- Minor Dental Procedures

The recommended dose is 90 mg once daily.

Doses greater than those recommended for each indication have either not demonstrated additional efficacy or have not been studied. Therefore:

The dose for OA should not exceed 60 mg daily.

The dose for RA should not exceed 90 mg daily.

The dose for ankylosing spondylitis should not exceed 90 mg daily.

The dose for acute gout should not exceed 120 mg daily.

The dose for acute pain and primary dysmenorrhea should not exceed 120 mg daily.

The dose for minor dental procedures should not exceed 90 mg daily.

Given the association between cardiovascular risk and exposure to COX-2 inhibitors, doctors are advised to use the lowest effective dose for the shortest possible duration of treatment. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically.

Elderly, Gender, Race

No dosage adjustment in ETROBAX is necessary for the elderly or based on gender or race.

Hepatic Insufficiency

In patients with mild hepatic insufficiency (Child-Pugh score 5-6), a dose of 60 mg once daily should not be exceeded. In patients with moderate hepatic insufficiency (Child-Pugh score 7-9), the dose should be reduced; a dose of 60 mg every other day should not be exceeded, administration of 30 mg once daily can also be considered. There are no clinical or pharmacokinetic data in patients with severe hepatic insufficiency (Child-Pugh score >9).

Renal Insufficiency

In patients with advanced renal disease (creatinine clearance <30 mL/min), treatment with ETROBAX is not recommended. No dosage adjustment is necessary for patients with lesser degrees of renal insufficiency (creatinine clearance ≥30 mL/min).

CONTRAINDICATIONS

Etoricoxib is contraindicated in patients with:

- Hypersensitivity to the active substance or to any of the excipients.
- Pregnancy.
- Congestive heart failure (NYHA II-IV).
- Hypertension whose blood pressure is persistently elevated above 140/90 mmHg and in patients with hypertension (high blood pressure) where blood pressure is not under control.
- Established ischaemic heart disease, peripheral arterial disease, and/or cerebrovascular disease (including patients who have recently undergone coronary artery bypass graft surgery or angioplasty) or increased risk of cardiovascular disease (ischaemic heart disease and stroke).

WARNINGS AND PRECAUTIONS

Risk of GI Ulceration, Bleeding and Perforation with NSAID

Serious GI toxicity such as bleeding, ulceration and perforation can occur at any time, with or without warning symptoms, in patients treated with NSAID therapy. Although minor upper GI problems (e.g. dyspepsia) are common, usually developing early in therapy, prescribers should remain alert for ulceration and *bleeding in patients treated with NSAIDs even in the absence of previous GI tract symptoms*.

Studies to date have not identified any subset of patients not at risk of developing peptic ulceration and bleeding. Patients with prior history of serious GI events and other risk factors associated with peptic ulcer disease (e.g. alcoholism, smoking, and corticosteroid therapy) are at increased risk. Elderly or debilitated patients seem to tolerate ulceration or bleeding less than other individuals and account for most spontaneous reports for fatal GI events.

Cardiovascular effects

It has been reported that the selective COX-2 inhibitor class of drugs may be associated with a risk of thrombotic events (especially myocardial infarction (MI) and stroke), relative to some NSAIDs. As the cardiovascular risks of etoricoxib may increase with dose and duration of exposure, the shortest duration possible and the lowest effective daily dose should be used. The patient's need for symptomatic relief and response to therapy should be re-evaluated periodically, especially in patients with osteoarthritis.

Warning to prescriber when prescribing COX-2 inhibitors to patients with risk factors of heart disease, hypertension (high blood pressure), hyperlipidemia, diabetes, smoking patient and patient with peripheral arterial disease.

Etoricoxib is contraindicated for patients who have increased risk of cardiovascular disease (ischaemic heart disease and stroke) [see CONTRAINDICATIONS].

COX-2 selective inhibitors are not a substitute for acetylsalicylic acid for prophylaxis of cardiovascular thrombo-embolic diseases because of their lack of antiplatelet effect. Therefore antiplatelet therapies should not be discontinued.

Renal effects

Long-term administration of NSAIDs has resulted in renal papillary necrosis and other renal injury. Renal prostaglandins may play a compensatory role in the maintenance of renal perfusion. Therefore, under conditions of compromised renal perfusion, administration of etoricoxib may cause a reduction in prostaglandin formation and, secondarily, in renal blood flow, and thereby impair renal function. Patients at greatest risk of this response are those with pre-existing significantly impaired renal function, uncompensated heart failure, or cirrhosis. Monitoring of renal function in such patients should be considered.

Fluid retention, oedema and hypertension

As with other medicinal products known to inhibit prostaglandin synthesis, fluid retention, oedema and hypertension have been reported in patients taking etoricoxib. All non-steroidal anti-inflammatory drugs (NSAIDs), including etoricoxib, can be associated with new onset or recurrent congestive heart failure. Caution should be exercised in patients with a history of cardiac failure, left ventricular dysfunction, or hypertension and in patients with pre-existing oedema from any other reason. If there is clinical evidence of deterioration in the condition of these patients, appropriate measures including discontinuation of etoricoxib should be taken.

Etoricoxib may be associated with more frequent and severe hypertension than some other NSAIDs and selective COX-2 inhibitors, particularly at high doses. Therefore, hypertension should be controlled before treatment with etoricoxib and special attention should be paid to blood pressure monitoring during treatment with etoricoxib. Blood pressure should be monitored within two weeks after initiation of treatment and periodically thereafter. If blood pressure rises significantly, alternative treatment should be considered.

Hepatic effects

Elevations of alanine aminotransferase (ALT) and/or aspartate aminotransferase (AST) (approximately three or more times the upper limit of normal) have been reported with etoricoxib 30, 60 and 90 mg daily. Any patients with symptoms and/or signs suggesting liver dysfunction, or in whom an abnormal liver function test has occurred, should be monitored. If signs of hepatic insufficiency occur, or if persistently abnormal liver function tests (three times the upper limit of normal) are detected, etoricoxib should be discontinued.

General

If during treatment, patients deteriorate in any of the organ system functions described above, appropriate measures should be taken and discontinuation of etoricoxib therapy should be considered. Medically appropriate supervision should be maintained when using etoricoxib in the elderly and in patients with renal, hepatic, or cardiac dysfunction.

Caution should be used when initiating treatment with etoricoxib in patients with dehydration. It is advisable to rehydrate patients prior to starting therapy with etoricoxib.

Serious skin reactions, some of them fatal, including exfoliative dermatitis, Stevens-Johnson syndrome, and toxic epidermal necrolysis, have been reported very rarely in association with the use of NSAIDs and some selective COX-2 inhibitors during post-marketing surveillance. Patients appear to be at highest risk for these reactions early in the course of therapy with the onset of the reaction occurring in the majority of cases within the first month of treatment. Serious hypersensitivity reactions (such as anaphylaxis and angioedema) have been reported in patients receiving etoricoxib. Some selective COX-2 inhibitors have been reported to be associated with an increased risk of skin reactions in

patients with a history of any drug allergy. Etoricoxib should be discontinued at the first appearance of skin rash, mucosal lesions, or any other sign of hypersensitivity.

Etoricoxib should be used with caution in patients who have previously experienced acute asthmatic attacks, urticaria, or rhinitis, which were precipitated by salicylates or non-selective cyclooxygenase inhibitors. Since the pathophysiology of these reactions is unknown, physicians should weigh the potential benefits of prescribing etoricoxib versus the potential risks.

Etoricoxib may mask fever and other signs of inflammation.

Caution should be exercised when co-administering etoricoxib with warfarin or other oral anticoagulants (see DRUG INTERACTIONS).

The use of etoricoxib, as with any medicinal product known to inhibit cyclooxygenase / prostaglandin synthesis, is not recommended in women attempting to conceive (see USE IN SPECIAL POPULATIONS).

Pediatric

Safety and effectiveness of etoricoxib in pediatric patients have not been established.

Elderly

Pharmacokinetics in the elderly (65 years of age and older) are reported to be similar to those in the young. A higher incidence of adverse experiences was reported in older patients compared to younger patients, the relative differences between etoricoxib and control group were similar in the elderly and the young. Greater sensitivity of some older individuals cannot be ruled out.

Effects on ability to drive and use machines

Patients who experience dizziness, vertigo or somnolence while taking etoricoxib should refrain from driving or operating machinery.

This product contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

DRUG INTERACTIONS

Pharmacodynamic interactions

Oral anticoagulants: The administration of etoricoxib 120 mg daily was reported to be associated with an increase in prothrombin time International Normalised Ratio (INR). Therefore, patients receiving oral

anticoagulants should be closely monitored for their prothrombin time INR, particularly in the first few days when therapy with etoricoxib is initiated or the dose of etoricoxib is changed.

Diuretics, ACE inhibitors and Angiotensin II Antagonists: NSAIDs may reduce the effect of diuretics and other antihypertensive drugs. In some patients with compromised renal function (e.g. dehydrated patients or elderly patients with compromised renal function) the co-administration of an ACE inhibitor or Angiotensin II antagonist and agents that inhibit cyclo-oxygenase may result in further deterioration of renal function, including possible acute renal failure, which is usually reversible. These interactions should be considered in patients taking etoricoxib concomitantly with ACE inhibitors or angiotensin II antagonists. Therefore, the combination should be administered with caution, especially in the elderly. Patients should be adequately hydrated and consideration should be given to monitoring of renal function after initiation of concomitant therapy, and periodically thereafter.

Acetylsalicylic Acid: It has been reported that etoricoxib 120 mg once daily had no effect on the anti-platelet activity of acetylsalicylic acid (81 mg once daily). Etoricoxib can be used concomitantly with acetylsalicylic acid at doses used for cardiovascular prophylaxis (low-dose acetylsalicylic acid). However, concomitant administration of low-dose acetylsalicylic acid with etoricoxib may result in an increased rate of GI ulceration or other complications compared to use of etoricoxib alone. Concomitant administration of etoricoxib with doses of acetylsalicylic acid above those for cardiovascular prophylaxis or with other NSAIDs is not recommended.

Ciclosporin and tacrolimus: Coadministration of ciclosporin or tacrolimus with any NSAID may increase the nephrotoxic effect of ciclosporin or tacrolimus. Renal function should be monitored when etoricoxib and either of these drugs is used in combination.

Pharmacokinetic interactions

The effect of etoricoxib on the pharmacokinetics of other drugs

Lithium: NSAIDs decrease lithium renal excretion and therefore increase lithium plasma levels. If necessary, monitor blood lithium closely and adjust the lithium dosage while the combination is being taken and when the NSAID is withdrawn.

Methotrexate: It has been reported that etoricoxib at 60 and 90 mg had no effect on methotrexate plasma concentrations or renal clearance. However, etoricoxib 120 mg was reported to increase methotrexate plasma concentrations and reduced renal clearance of methotrexate. Adequate monitoring for methotrexate-related toxicity is recommended when etoricoxib and methotrexate are administered concomitantly.

Oral contraceptives: Etoricoxib 60 mg given concomitantly with an oral contraceptive containing 35 micrograms ethinyl estradiol (EE) and 0.5 to 1 mg norethindrone for 21 days has been reported to increase the steady state AUC_{0-24hr} of EE. Etoricoxib 120 mg given with the same oral contraceptive concomitantly or separated by 12 hours, was reported to increase the steady state AUC_{0-24hr} of EE. This increase in EE concentration should be considered when selecting an oral contraceptive for use with etoricoxib. An increase in EE exposure can increase the incidence of adverse events associated with oral contraceptives (e.g., venous thrombo-embolic events in women at risk).

Hormone Replacement Therapy (HRT): Administration of etoricoxib 120 mg with hormone replacement therapy consisting of conjugated estrogens (0.625 mg conjugated estrogens) for 28 days, has been reported to increase the mean steady state AUC_{0-24hr} of unconjugated estrone, equilin, and 17- β -estradiol. The effect of the recommended chronic doses of etoricoxib (30, 60, and 90 mg) has not been studied. The effects of etoricoxib 120 mg on the exposure (AUC_{0-24hr}) to these estrogenic components of conjugated estrogens were less than half of those reported when conjugated estrogens was administered alone and the dose was increased from 0.625 to 1.25 mg. The clinical significance of these increases is unknown, and higher doses of conjugated estrogens were not studied in combination with etoricoxib. These increases in estrogenic concentration should be taken into consideration when selecting post-menopausal hormone therapy for use with etoricoxib because the increase in oestrogen exposure might increase the risk of adverse events associated with HRT.

Prednisone/prednisolone: Etoricoxib did not have clinically important effects on the pharmacokinetics of prednisone/prednisolone.

Digoxin: Etoricoxib 120 mg administered once daily for 10 days did not alter the steady-state plasma AUC_{0-24hr} or renal elimination of digoxin. There was an increase in digoxin C_{max} . This increase is not generally important for most patients. However, patients at high risk of digoxin toxicity should be monitored for this when etoricoxib and digoxin are administered concomitantly.

Effect of etoricoxib on drugs metabolised by sulfotransferases

Etoricoxib is an inhibitor of human sulfotransferase activity, particularly SULT1E1, and has been reported to increase the serum concentrations of ethinyl estradiol. While knowledge about effects of multiple sulfotransferases is presently limited, it may be prudent to exercise care when administering etoricoxib concurrently with other drugs primarily metabolised by human sulfotransferases (e.g., oral salbutamol and minoxidil).

Effect of etoricoxib on drugs metabolised by CYP isoenzymes

Based on reported information, etoricoxib is not expected to inhibit cytochromes P450 (CYP) 1A2, 2C9, 2C19, 2D6, 2E1 or 3A4. It was reported that daily administration of etoricoxib 120 mg did not alter hepatic CYP3A4 activity.

Effects of other drugs on the pharmacokinetics of etoricoxib

The main pathway of etoricoxib metabolism is dependent on CYP enzymes. CYP3A4 appears to contribute to the metabolism of etoricoxib *in vivo*. It is reported that CYP2D6, CYP2C9, CYP1A2 and CYP2C19 also can catalyse the main metabolic pathway, but their quantitative roles have not been studied *in vivo*.

Ketoconazole: Ketoconazole, a potent inhibitor of CYP3A4, dosed at 400 mg once a day for 11 days reportedly did not have any clinically important effect on the single-dose pharmacokinetics of 60 mg etoricoxib.

Voriconazole and Miconazole: Co-administration of either oral voriconazole or topical miconazole oral gel, strong CYP3A4 inhibitors, with etoricoxib reportedly caused a slight increase in exposure to etoricoxib, but is not considered to be clinically meaningful based on published information.

Rifampicin: It has been reported that co-administration of etoricoxib with rifampicin, a potent inducer of CYP enzymes, produced a decrease in etoricoxib plasma concentrations. This interaction may result in recurrence of symptoms when etoricoxib is co-administered with rifampicin. While this information may suggest an increase in dose, doses of etoricoxib greater than those listed for each indication have not been studied in combination with rifampicin and are therefore not recommended.

Antacids: Antacids do not affect the pharmacokinetics of etoricoxib to a clinically relevant extent.

SIDE EFFECTS/UNDESIRABLE EFFECTS

The adverse event profile of etoricoxib was similar in patients with OA, RA, chronic low back pain, acute gouty arthritis or in patients with acute postoperative dental pain following surgery.

The following undesirable effects were reported with etoricoxib.

System organ class	Adverse reactions	Frequency category
Infections and infestations	alveolar osteitis	Common
	gastroenteritis, upper respiratory infection, urinary tract infection	Uncommon
Blood and lymphatic system disorders	anaemia (primarily associated with gastrointestinal bleeding), leukopenia, thrombocytopenia	Uncommon
Immune system disorders	hypersensitivity [§]	Uncommon
	angioedema/anaphylactic /anaphylactoid reactions including shock	Rare

System organ class	Adverse reactions	Frequency category
<i>Metabolism and nutrition disorders</i>	oedema/fluid retention	Common
	appetite increase or decrease, weight gain	Uncommon
<i>Psychiatric disorders</i>	anxiety, depression, mental acuity decreased, hallucinations	Uncommon
	confusion, restlessness	Rare
<i>Nervous system disorders</i>	dizziness, headache	Common
	dysgeusia, insomnia, paresthaesia/hypaesthesia, somnolence	Uncommon
<i>Eye disorders</i>	blurred vision, conjunctivitis	Uncommon
<i>Ear and labyrinth disorders</i>	tinnitus, vertigo	Uncommon
<i>Cardiac disorders</i>	palpitations, arrhythmia	Common
	atrial fibrillation, tachycardia, congestive heart failure, non-specific ECG changes, angina pectoris, myocardial infarction ^s	Uncommon
<i>Vascular disorders</i>	hypertension	Common
	flushing, cerebrovascular accident ^s , transient ischaemic attack, hypertensive crisis, vasculitis	Uncommon
<i>Respiratory, thoracic and mediastinal disorders</i>	bronchospasm	Common
	cough, dyspnoea, epistaxis	Uncommon
<i>Gastrointestinal disorders</i>	abdominal pain	Very common
	constipation, flatulence, gastritis, heartburn/acid reflux, diarrhea, dyspepsia/epigastric discomfort, nausea, vomiting, oesophagitis, oral ulcer	Common
	abdominal distention, bowel movement pattern change, dry mouth, gastroduodenal ulcer, peptic ulcers including gastrointestinal perforation and bleeding, irritable bowel syndrome, pancreatitis	Uncommon
<i>Hepatobiliary disorders</i>	ALT increased, AST increased	Common
	hepatitis	Rare
	hepatic failure, jaundice	Rare
<i>Skin and subcutaneous tissue disorders</i>	ecchymosis	Common
	facial oedema, pruritus, rash, erythema,	Uncommon

System organ class	Adverse reactions	Frequency category
	urticaria	
	Stevens-Johnson syndrome, toxic epidermal necrolysis, fixed drug eruption	Rare
<i>Musculoskeletal and connective tissue disorders</i>	muscular cramp/spasm, musculoskeletal pain/stiffness	Uncommon
<i>Renal and urinary disorders</i>	proteinuria, serum creatinine increased, renal failure/renal insufficiency	Uncommon
<i>General disorders and administration site conditions</i>	asthenia/fatigue, flu-like disease	Common
	chest pain	Uncommon
<i>Investigations</i>	blood urea nitrogen increased, creatine phosphokinase increased, hyperkalaemia, uric acid increased	Uncommon
	blood sodium decreased	Rare

[§] Hypersensitivity includes the terms "allergy", "drug allergy", "drug hypersensitivity", "hypersensitivity", "hypersensitivity NOS", "hypersensitivity reaction" and "nonspecific allergy".

[§] Selective COX-2 inhibitors have been reported to be associated with an increased risk of serious thrombotic arterial events, including myocardial infarction and stroke.

The following serious undesirable effects have been reported in association with the use of NSAIDs and cannot be ruled out for etoricoxib: nephrotoxicity including interstitial nephritis and nephrotic syndrome.

USE IN SPECIAL POPULATIONS

• Pregnancy

No clinical data on exposed pregnancies are available for etoricoxib. Reproductive toxicity has been reported in animals. The potential for human risk in pregnancy is unknown. Etoricoxib, as with other medicinal products inhibiting prostaglandin synthesis, may cause uterine inertia and premature closure of the ductus arteriosus during the last trimester. Etoricoxib is contraindicated in pregnancy (see CONTRAINDICATIONS). If a woman becomes pregnant during treatment, etoricoxib must be discontinued.

Fertility

The use of etoricoxib, as with any drug substance known to inhibit COX-2, is not recommended in women attempting to conceive.

- **Lactation**

Etoricoxib is reported to be excreted in the milk of lactating rats. It is not known whether this drug is excreted in human milk. Because many drugs are excreted in human milk and because of the possible adverse effects of drugs that inhibit prostaglandin synthesis on nursing infants, a decision should be made whether to discontinue nursing or to discontinue the drug, taking into account the importance of the drug to the mother.

OVERDOSE

It has been reported that administration of single doses of etoricoxib up to 500 mg and multiple doses up to 150 mg/day for 21 days did not result in significant toxicity. There have been reports of acute overdosage with etoricoxib, although adverse experiences were not reported in the majority of cases. The most frequently reported adverse experiences were consistent with the safety profile for etoricoxib (e.g. gastrointestinal events, cardiorenal events).

In the event of overdose, it is reasonable to employ the usual supportive measures, e.g., remove unabsorbed material from the GI tract, employ clinical monitoring, and institute supportive therapy, if required. Etoricoxib is not dialysable by haemodialysis; it is not known whether etoricoxib is dialysable by peritoneal dialysis.

STORAGE

Store below 30°C, in original pack

DOSAGE FORM AND PACKAGING AVAILABLE

Cold Form Blister Pack of 7's Tablets/box

Date of Revision: October 2019

Manufactured in India by
Sun Pharmaceutical Industries Limited
Village Ganguwala,
Paonta Sahib District Sirmour - 173 025,
Himanchal Pradesh, INDIA

Product Registration Holder:
RANBAXY (MALAYSIA) SDN. BHD.

(A Sun Pharma Company)
Lot 23, Bakar Arang Industrial Estate,
08000 Sunagi Petani, Kedah, Malaysia